Ayushman Bharat PM-JAY

Alternative Question Bank — 100+ Questions

6 Thinking Styles | 3 Stages | Q&A Format

Consequence • Decision • Process • Fraud & Security • Reasoning • Analysis

For Students, Researchers & Ayushman Mitras

STAGE 1 – Eligibility & Identification

What Happens If... (Consequence Based)

Q1. What happens if an eligible family never checks the PM-JAY portal and doesn't register?

➤ They remain unregistered and cannot access cashless treatment. During a medical emergency, they will have to pay out-of-pocket — losing a benefit they are legally entitled to. The ₹5 lakh annual cover goes completely unused.

Q2. What happens if a family's eligibility data in SECC 2011 is wrong — like wrong name or wrong village?

➤ They may fail the eligibility search even though they are genuinely eligible. Minor spelling mismatches can be resolved via Aadhaar-based verification. Major errors need a grievance at the district health office or helpline 14555.

Q3. What happens if a state government refuses to implement PM-JAY?

➤ Eligible families in that state lose access to the central scheme's benefits. The state must either run its own equivalent scheme or leave beneficiaries unprotected. Politically, this creates public accountability pressure on the state government.

Q4. What happens to the ₹5 lakh limit if a family does not use it in a year?

➤ The unused amount does not carry forward. The ₹5 lakh limit resets fresh every year — it is a 'use it or lose it' annual benefit, not a cumulative savings pool.

Q5. What happens if a person is found eligible but refuses to enrol?

➤ They simply won't get the card and can't use cashless treatment. There is no penalty for not enrolling — it is voluntary. However, they lose access to ₹5 lakh worth of free healthcare every year.

Q6. What happens if SECC 2011 data shows a family as eligible but they have since become financially well-off?

➤ Currently, there is no automatic mechanism to remove them. Once in the SECC database, they remain eligible unless the government updates the database. This is called an 'inclusion error' and is a known limitation of using static 2011 data.

Q7. What happens to migrants whose home state data shows them eligible but they are now living in another state?

➤ They are still eligible and can use PM-JAY in their new state since the scheme is portable across all of India. They don't need to transfer their registration — just show the Ayushman Card at any empanelled hospital.

Q8. What happens if a person's Aadhaar is linked to the wrong family in SECC data?

➤ Their eKYC will link them to the wrong family record, causing confusion over the family's ₹5 lakh limit. The fix requires visiting the district health office to manually correct the family-Aadhaar linkage in the system.

What Will I Do If... (Decision Based)

Q9. What will I do if my name doesn't appear when I search on the PM-JAY app?

➤ First try alternative search methods — use ration card number or name with location. If still not found, visit the nearest CSC or district health office and ask for a grievance to be filed. Call 14555 for assistance. Don't assume you're ineligible without verifying in person.

Q10. What will I do if my family has moved to a new city but our SECC data shows the old village address?

➤ Nothing needs to change for PM-JAY. The scheme is fully portable. Go to any empanelled hospital in your new city, show your Ayushman Card and Aadhaar, and avail cashless treatment without updating your address.

Q11. What will I do if I belong to an urban occupational category but I'm not in SECC 2011 data?

➤ Visit your district health office or SHA office and explain your situation. Many states have extended PM-JAY coverage beyond SECC data for specific occupational groups. Your state may have a process to add such individuals through a state-level beneficiary list.

Q12. What will I do if I suspect my neighbour is using a forged Aadhaar to claim PM-JAY benefits?

➤ Report it to the PM-JAY fraud helpline or via pmjay.gov.in's grievance portal. NHA has a dedicated fraud monitoring unit. Providing details like the person's name and hospital where misuse happened will help authorities investigate.

Q13. What will I do if my elderly mother has no Aadhaar and is hospitalised urgently? Can she still get PM-JAY benefits?

➤ Without Aadhaar, eKYC cannot be completed for a new registration. However, if another family member already has an Ayushman Card that covers the family, she can be treated under that card. In parallel, initiate emergency Aadhaar enrolment for her.

Q14. What will I do if the hospital tells me I'm not eligible even though I have an Ayushman Card?

➤ Insist the hospital verify through the PM-JAY portal — the card's validity is confirmed online, not by the hospital staff's opinion. If they still refuse, call 14555 immediately. Wrongful denial by hospitals is a punishable offence under the scheme.

How To Go Through... (Process Based)

Q15. How do I go through the process of confirming my family's PM-JAY eligibility from scratch?

➤ Step 1: Open pmjay.gov.in or the Ayushman Bharat app. Step 2: Enter your mobile number and verify with OTP. Step 3: Search using Aadhaar number, ration card number, or name + district. Step 4: If your family appears, you're eligible. Step 5: Proceed to eKYC to generate your card. If not found, visit CSC or call 14555.

Q16. How does the government go through the process of identifying eligible families across millions of households?

➤ The government uses SECC 2011 census data collected by enumerators who visited every household. This data was digitised and cross-verified with ration card databases. The resulting list of

eligible families was loaded into the PM-JAY beneficiary system and seeded with Aadhaar numbers for verification.

Q17. How does a state go through the process of expanding PM-JAY beyond SECC data?

➤ The state SHA defines new eligible categories through a government order, creates a state beneficiary list, funds the expansion entirely from state budget, seeds Aadhaar for new beneficiaries, uploads the list to the NHA portal, and begins registration for the newly added families.

What If Someone Misuses... (Fraud & Security Angle)

Q18. What if someone uses a forged Aadhaar card with a real-looking photo to claim they are eligible for PM-JAY?

➤ The Aadhaar verification system uses biometric fingerprint, iris, or face authentication — not just the card's appearance. A forged physical card will fail at biometric verification since the imposter's fingerprints won't match the UIDAI database. The system will reject the verification and flag the attempt.

Q19. What if a family that has become financially stable continues to use PM-JAY benefits by not disclosing their improved status?

➤ Currently there is no automatic income re-verification mechanism. However, future SECC updates or integration with income tax/GST databases could flag such families. Intentional misuse once discovered can lead to recovery of costs and legal action under fraud provisions.

Q20. What if a fraudster creates a fake family by mixing real and fake Aadhaar numbers to get a PM-JAY Family ID?

➤ Each Aadhaar must pass biometric verification individually during eKYC. Fake Aadhaar numbers will either fail the UIDAI validation or fail biometric matching. The system cross-checks each Aadhaar against the SECC family record, making random mixing of fake and real numbers detectable.

Q21. What if hospital staff and a fraudster collude to enrol a fake 'family' in the system to generate multiple ₹5 lakh limits?

➤ NHA's AI fraud detection monitors claim patterns. Multiple new registrations from the same hospital in a short period, or claims filed immediately after registration, trigger red flags. The hospital faces audit, potential delisting, and criminal proceedings. The fraudster faces prosecution under IPC fraud sections.

Why Does This Exist... (Reasoning Based)

Q22. Why does PM-JAY use SECC 2011 data instead of letting anyone apply and self-declare their poverty?

➤ Self-declaration would open the scheme to massive misuse by non-poor families claiming eligibility. SECC 2011 provides an independent, government-collected baseline of genuinely poor households, making the targeting more reliable even if imperfect due to its age.

Q23. Why does PM-JAY cover only hospitalisation and not OPD (outpatient) treatment?

➤ Including OPD would multiply the cost and administrative burden enormously — most doctor visits are outpatient. By focusing on hospitalisation, PM-JAY protects families from the most catastrophic health expenditures (surgeries, ICU care) that can push them into poverty.

Q24. Why is the PM-JAY benefit set at ₹5 lakh per family per year and not per individual?

➤ A per-family limit controls total government expenditure while ensuring the benefit is meaningful for the most critical needs. Most hospitalisation costs fall well below ₹5 lakh per event. Per-individual limits would be exploited through multiple small claims.

Q25. Why does PM-JAY include both government and private hospitals instead of only government hospitals?

➤ Government hospitals alone cannot absorb the treatment demand from 10+ crore families. Including empanelled private hospitals dramatically expands the network of available care, reduces travel burden for rural beneficiaries, and increases competition that can improve quality.

Compare This vs That... (Analysis Based)

Q26. Compare PM-JAY's eligibility model with Tamil Nadu's CM Comprehensive Health Insurance Scheme.

➤ PM-JAY targets SECC 2011 families nationally with ₹5 lakh cover. Tamil Nadu's CMCHIS covers families with annual income below ₹72,000 with up to ₹5 lakh cover and has broader eligibility than SECC alone. CMCHIS is state-funded for its expanded population while PM-JAY is Centre-State shared. Tamil Nadu integrates both schemes for maximum coverage.

Q27. Compare India's PM-JAY targeted approach vs UK's NHS universal approach for health coverage.

➤ PM-JAY: targets only poor families, means-tested eligibility, co-funded by Centre and State, covers hospitalisation only. NHS (UK): covers every citizen regardless of income, funded entirely by taxes, covers both OPD and IPD. PM-JAY is cheaper to run but risks exclusion errors. NHS is comprehensive but requires very high tax revenue — suitable for a richer economy.

Q28. Compare eligibility under PM-JAY for a rural family vs an urban family.

➤ Rural: eligibility based on SECC 2011 deprivation criteria (no shelter, manual scavenging, tribal status etc.). Urban: eligibility based on occupational categories (construction worker, rag picker, domestic worker etc.). Both use SECC as the base. Rural criteria are more deprivation-focused while urban criteria are more occupation-focused.

STAGE 2 – Registration & Card Generation

What Happens If... (Consequence Based)

Q1. What happens if a person skips eKYC and tries to use a downloaded Ayushman Card at the hospital?

➤ The hospital will verify the card through the PM-JAY portal in real time, which requires a live biometric or OTP confirmation at the time of admission. Without completed eKYC, the card won't pass hospital-side verification and the patient will be turned away.

Q2. What happens if someone loses their Ayushman Card before using it?

➤ Nothing is permanently lost. The Ayushman Card is a digital card and can be re-downloaded anytime from the Ayushman Bharat app or beneficiary.nha.gov.in using the registered mobile OTP. A new physical printout can also be obtained at any CSC.

Q3. What happens if a person registers successfully but uses a wrong mobile number during sign-up?

➤ They won't receive future OTPs or SMS updates on that number. To fix it, they should visit a CSC or district health office to update their registered mobile number. Without the correct number, accessing the card digitally becomes difficult.

Q4. What happens if two people from the same family do eKYC separately on different devices at the same time?

➤ The system assigns them to the same Family ID. Both verifications will link to the same family record. This is expected and acceptable — family members can individually complete eKYC and download their individual cards linked to the same Family ID.

Q5. What happens if the CSC operator makes an error while entering family details during registration?

➤ The family should immediately check the card details after generation. If errors are found, visit the CSC or district health office with correct documents and request a correction. Aadhaar-linked details (name, date of birth) can only be corrected at the Aadhaar level first.

Q6. What happens if a beneficiary's Aadhaar is locked when they try to do eKYC for PM-JAY?

➤ eKYC will fail because Aadhaar verification requires an unlocked Aadhaar. The person must first unlock their Aadhaar via the UIDAI website or mAadhaar app, then re-attempt the PM-JAY eKYC.

Q7. What happens if a newborn is not added to the family's Ayushman Card after birth?

➤ The newborn won't have individual PM-JAY coverage until added. However, since the scheme covers the entire family unit, the newborn may still be treated under the family card in practice. For formal coverage, the parents should visit a CSC to add the newborn to the family record.

Q8. What happens if the Ayushman Card expires or becomes outdated?

➤ The Ayushman Card does not have an expiry date. It remains valid as long as the family is in the beneficiary database. The ₹5 lakh benefit refreshes every year automatically — the card itself doesn't need renewal.

What Will I Do If... (Decision Based)

Q9. What will I do if my fingerprint fails repeatedly during biometric verification at the CSC?

➤ Request to switch to face authentication (live photo via app camera) or OTP-based eKYC if your mobile is linked to Aadhaar. If all methods fail, the CSC operator can raise a technical support ticket with NHA. Manual verification with additional documents may be arranged at the district health office.

Q10. What will I do if I registered successfully on the app but the card is not generating even after 2 days?

➤ Check your internet connection and retry downloading. If still failing: visit the nearest CSC to regenerate, call helpline 14555 for technical support, or file a grievance on pmjay.gov.in. Server-side delays sometimes occur during large-scale registration drives.

Q11. What will I do if I want to add my newly married daughter-in-law to the family's Ayushman Card?

➤ Visit the nearest CSC or district health office with her Aadhaar card and the family's ration card showing her addition. Request family member update in the PM-JAY system. She will then need to complete her individual eKYC to generate her personal Ayushman Card under the same Family ID.

Q12. What will I do if I'm a homeless person with no fixed address — can I still register for PM-JAY?

➤ Yes. A permanent address is not mandatory. You can use the address of a shelter home, NGO, or a trusted community contact. Identity verification is Aadhaar-based, so as long as you have Aadhaar, registration is possible. Ayushman Mitras at government hospitals can assist homeless individuals.

Q13. What will I do if my elderly father cannot come to the CSC physically for registration?

➤ You can register him as part of the family using your Family ID. His individual Ayushman Card can be generated once — the family head's registration covers him for treatment. For his individual card, a mobile Ayushman Mitra or camp visit to his location can be arranged through the district health office.

Q14. What will I do if I suspect my neighbour registered using my family's SECC record?

➤ Check your family's PM-JAY record online at beneficiary.nha.gov.in to see all registered members. If an unauthorised person has been added, report immediately to 14555 and file a grievance. NHA can trace registrations by Aadhaar and remove unauthorised entries.

How To Go Through... (Process Based)

Q15. How do I go through the complete online registration process if I have a smartphone and Aadhaar-linked mobile?

➤ Step 1: Download the Ayushman Bharat app. Step 2: Login with mobile number + OTP. Step 3: Search eligibility using Aadhaar or ration card number. Step 4: If eligible, tap 'Get Card'. Step 5: Complete eKYC via OTP (sent to Aadhaar-linked mobile) or face authentication. Step 6: Confirm family details. Step 7: Card is generated — download and save it.

Q16. How does the hospital go through the process of verifying my Ayushman Card when I arrive?

➤ Step 1: You show your Ayushman Card + Aadhaar at the hospital's PM-JAY desk. Step 2: The Ayushman Mitra at the hospital logs into the PM-JAY portal. Step 3: They enter your details and initiate live verification — OTP to your phone or biometric scan. Step 4: System confirms eligibility and remaining ₹5 lakh balance. Step 5: Cashless treatment authorisation is generated.

Q17. How does the government go through the process of ensuring every registered beneficiary actually has a valid card?

➤ NHA runs periodic verification drives — checking that all registered Aadhaar numbers are active, cards are downloaded, and families are reachable. State Health Agencies conduct ground-level beneficiary verification camps. AI analytics flag dormant registrations for follow-up outreach.

Q18. How do I go through the process of correcting a mistake on my Ayushman Card?

➤ Step 1: Identify the error (name, date of birth etc.). Step 2: If it's an Aadhaar-linked detail, first correct it at UIDAI (uidai.gov.in or Aadhaar centre). Step 3: Once Aadhaar is corrected, re-do eKYC on the PM-JAY app — the updated details will auto-populate. Step 4: Download the corrected card.

What If Someone Misuses... (Fraud & Security Angle)

Q19. What if someone steals my Aadhaar card and tries to register for PM-JAY using my identity?

➤ They cannot complete eKYC without matching your biometric fingerprint, iris, or face. The PM-JAY system doesn't accept just the Aadhaar card number — live biometric or OTP to your registered mobile is required. As a precaution, you can lock your Aadhaar biometric on uidai.gov.in to block all biometric-based use.

Q20. What if a CSC operator fraudulently registers fake family members to inflate the family's coverage?

➤ Each member added must complete individual eKYC with their own Aadhaar and biometrics. Fake members without real Aadhaar cannot pass eKYC. NHA audits CSC registration activity and flags unusual patterns — like a single CSC registering hundreds of new members in a day.

Q21. What if someone generates an Ayushman Card in my name without my knowledge by accessing my Aadhaar number?

➤ This is practically impossible without your biometric or OTP consent. However, if you suspect misuse, check your registration status at beneficiary.nha.gov.in and contact 14555. You can also lock your Aadhaar biometric on the UIDAI portal to prevent further unauthorised use.

Q22. What if a person colludes with a hospital and registers for PM-JAY multiple times under slightly different names using the same Aadhaar?

➤ The Aadhaar deduplication engine will detect and block duplicate registrations linked to the same Aadhaar number regardless of name variations. The Aadhaar number is the unique key — the same Aadhaar cannot be linked to two different Family IDs simultaneously.

Q23. What if someone hacks the PM-JAY portal and changes a family's registered details to redirect benefits?

➤ PM-JAY uses Aadhaar-based authentication — any change to beneficiary details requires biometric verification by the actual person. Hacking the portal without Aadhaar authentication cannot alter verified beneficiary records. NHA conducts regular security audits and penetration testing of the portal.

Why Does This Exist... (Reasoning Based)

Q24. Why is eKYC mandatory for getting the Ayushman Card instead of just submitting documents?

➤ Physical documents can be forged. eKYC ties the card to the person's biometric identity (fingerprint, face, or Aadhaar OTP) which is nearly impossible to fake. This prevents a card from being used by someone other than the actual beneficiary at the hospital.

Q25. Why does PM-JAY use Aadhaar as the backbone for registration instead of ration cards or voter IDs?

➤ Aadhaar is biometrically linked to the individual — it's the only government ID that proves 'you are who you say you are' through fingerprint or face scan. Ration cards and voter IDs are paper-based and can be forged or shared. Aadhaar ensures one person = one identity.

Q26. Why is the Ayushman Card digital-first instead of only a physical card?

➤ A digital card can be re-downloaded instantly if lost, is harder to forge than paper, can be verified in real time at the hospital through the portal, and eliminates printing/distribution costs for crores of beneficiaries. Physical cards are still available as a convenience option for those without smartphones.

Q27. Why does PM-JAY allow face authentication in addition to fingerprint — why not stick to one method?

➤ Fingerprints degrade in elderly people and manual labourers due to wear, cuts, and skin conditions — causing frequent failures. Face authentication provides an alternative that works reliably for these populations. Multiple methods ensure no eligible beneficiary is excluded due to technical limitations of a single method.

Compare This vs That... (Analysis Based)

Q28. Compare OTP-based eKYC vs biometric eKYC vs face authentication for PM-JAY registration.

➤ OTP-based: fastest, needs Aadhaar-linked mobile, can be done on personal phone, fails if mobile not linked. Biometric: needs CSC with fingerprint/iris device, works without mobile linkage, fails for elderly/labourers with worn fingerprints. Face authentication: works on smartphone camera, most inclusive, doesn't need CSC device, requires good lighting. Best choice depends on the beneficiary's situation.

Q29. Compare registration at a CSC vs registration via the app — which is better for whom?

➤ CSC: better for elderly, non-smartphone users, those with Aadhaar mobile linkage issues, provides physical card printout. App: better for tech-savvy individuals with Aadhaar-linked mobile, faster, self-service, no travel needed, instant digital card download. Both lead to the same Ayushman Card — the method depends on the beneficiary's access and comfort.

Q30. Compare PM-JAY card registration complexity with getting a regular private health insurance policy.

➤ PM-JAY: free, no medical tests, no premium, no rejection, covers pre-existing conditions from day one, biometric-based, simple process. Private insurance: paid premium, medical underwriting, can reject applicants with pre-existing conditions, complex paperwork, agent involvement. PM-JAY is far simpler and more inclusive, though limited to government-defined health packages unlike flexible private policies.

STAGE 3 – Awareness & Outreach

What Happens If... (Consequence Based)

Q1. What happens if awareness campaigns are conducted only once and never followed up?

➤ Registration rates improve initially but quickly plateau. New eligible families (newborns, newly poor, migrants) remain unaware. People who registered but never used the card forget about it. Research consistently shows one-time campaigns have low long-term impact — continuous outreach is essential for sustained utilisation.

Q2. What happens if outreach is only done in dominant regional languages and ignores tribal languages?

➤ Tribal communities remain unreached and unregistered. They continue paying out-of-pocket for hospitalisation or avoiding treatment altogether. Exclusion of linguistic minorities from awareness is a key reason for low PM-JAY utilisation among the most marginalised — the very people the scheme most needs to reach.

Q3. What happens if Ayushman Mitras at hospitals are poorly trained or absent?

➤ Eligible patients arriving at hospitals don't get guided through the verification process, face confusion, or are turned away. Poor Mitra performance directly translates to underutilisation — the scheme exists but patients can't access it effectively.

Q4. What happens if a beneficiary goes to an empanelled hospital but no one there knows about PM-JAY?

➤ They end up paying out-of-pocket despite being entitled to free treatment. This is a real problem in newly empanelled hospitals with untrained staff. Outreach must include hospital-side awareness training — not just community-side registration drives.

Q5. What happens when outreach focuses only on registration numbers and ignores utilisation?

➤ The state shows impressive card generation statistics but low actual treatment rates. Beneficiaries have cards but don't use them because they don't know what's covered, which hospitals are empanelled, or how to navigate the admission process. Coverage numbers look good on paper while actual health outcomes don't improve.

Q6. What happens if misinformation spreads in a community that PM-JAY requires upfront payment?

➤ Eligible families avoid hospitals even during emergencies, continuing to suffer financially and health-wise. Correcting misinformation requires active outreach — community meetings, ASHA worker counselling, and sharing real stories of beneficiaries who received free treatment.

What Will I Do If... (Decision Based)

Q7. What will I do if I am an outreach worker and a villager refuses to believe PM-JAY is real and free?

➤ Don't argue — demonstrate. Open the app together, search their name, show their family record. Offer to take them to the nearest empanelled hospital to see the PM-JAY desk. Share a contact of a neighbour or community member who has already used the card successfully. Real proof beats explanation every time.

Q8. What will I do if I'm organising an outreach camp in a village and the internet goes down mid-camp?

➤ Use the app's offline mode for basic eligibility checks if available. Record eligible family details manually on paper for entry later. Focus on awareness and document collection during the downtime. Schedule a follow-up date for completing digital registration once connectivity is restored.

Q9. What will I do as an Ayushman Mitra if a patient at the hospital is eligible but panicking and can't remember their Aadhaar number?

➤ Calm them first. Help them search their name on the PM-JAY portal using name + date of birth + district. Check if a family member has the Aadhaar number. If the situation is urgent and treatment can't wait, coordinate with the hospital's medical team to begin treatment while verification is completed — hospitals are instructed not to delay emergency care.

Q10. What will I do if I discover that a local hospital empanelled under PM-JAY is secretly charging patients cash?

➤ Document evidence (receipts, patient testimonies). Report immediately to the district SHA office and the PM-JAY helpline 14555. File a formal complaint at pmjay.gov.in. NHA can suspend and delist the hospital pending investigation. Publicise the grievance channel to other affected patients.

Q11. What will I do if I'm tasked with increasing PM-JAY awareness in an urban slum area where people are highly mobile and distrust government schemes?

➤ Partner with trusted local figures — community leaders, teachers, local shop owners. Set up awareness stalls at locations people visit daily — ration shops, bus stops, markets. Use simple visual posters in the local language. Focus the message on 'it's already yours — you just need to claim it'. Conduct on-spot registration to reduce the action barrier.

Q12. What will I do if the outreach data shows a particular block has 90% registration but only 5% utilisation?

➤ Shift outreach focus from registration to utilisation. Conduct 'How to Use Your Card' workshops. Distribute printed lists of empanelled hospitals. Train ASHA workers to accompany first-time PM-JAY patients to hospital. Survey cardholders to understand specific barriers — distance, fear, lack of information about covered treatments.

How To Go Through... (Process Based)

Q13. How does an outreach worker go through the process of planning and executing a village-level PM-JAY awareness camp?

➤ Step 1: Get list of unregistered eligible families from the SHA dashboard. Step 2: Coordinate with gram panchayat and fix a date/venue. Step 3: Arrange CSC operator with biometric device and laptop. Step 4: Print IEC materials in local language. Step 5: Conduct gram sabha — explain benefits in simple terms. Step 6: Run one-on-one registrations. Step 7: Hand out printed cards. Step 8: Share 14555 for follow-up. Step 9: Report camp outcomes to district SHA.

Q14. How does the government go through the process of tracking whether awareness campaigns are actually working?

➤ NHA monitors registration rates by district and block on a real-time dashboard. SHA conducts periodic awareness surveys measuring beneficiary knowledge. Claim data shows utilisation rates. Grievance volumes indicate whether people are facing problems. Independent research organisations conduct household surveys. All these inputs are synthesised into state-wise performance reports.

Q15. How does an Ayushman Mitra go through the process of helping a first-time PM-JAY patient navigate hospital admission?

➤ Step 1: Greet the patient at the hospital's PM-JAY help desk. Step 2: Verify their Ayushman Card and Aadhaar. Step 3: Check their condition against covered health packages on the portal. Step 4: Initiate pre-authorisation if required for the procedure. Step 5: Coordinate with the treating doctor and billing

department to ensure cashless processing. Step 6: Explain what is covered and what (if anything) is not. Step 7: Follow up post-discharge to ensure no unauthorised billing occurred.

Q16. How does the state go through the process of conducting a PM-JAY saturation drive in an entire district?

➤ Step 1: Pull list of all unregistered eligible families district-wide. Step 2: Divide into block-wise targets. Step 3: Deploy additional Ayushman Mitras and CSC operators. Step 4: Coordinate with sarpanch, school teachers, ASHA workers for community mobilisation. Step 5: Run daily camps across all villages simultaneously. Step 6: Track registration progress on live dashboard. Step 7: Daily review meetings to redirect resources to lagging areas. Step 8: Declare saturation when 95%+ families are registered.

What If Someone Misuses... (Fraud & Security Angle)

Q17. What if a fraudster poses as an Ayushman Mitra and asks villagers for money to register them for PM-JAY?

➤ PM-JAY registration is completely free — any demand for money is a fraud. Villagers should refuse and report immediately to 14555 or the district health office. Real Ayushman Mitras carry official ID cards and never ask for payment. Outreach programs must proactively warn communities about registration scams.

Q18. What if a fake 'PM-JAY camp' is set up in a village to collect Aadhaar details and OTPs from villagers for identity theft?

➤ This is a serious crime. Outreach workers should educate communities to verify camps through the official panchayat notice and call 14555 to confirm. Real PM-JAY representatives never ask for OTPs — OTPs are only used by the beneficiary themselves during eKYC. Anyone asking for your OTP is a fraudster.

Q19. What if an NGO claims to offer 'faster PM-JAY registration' for a fee and collects money from eligible families?

➤ This is illegal. PM-JAY registration has no fast-track for money — the process is the same for everyone and is free. Families should refuse, report to 14555, and register through official CSC channels. Outreach must create strong community awareness that NO FEE is ever required for PM-JAY.

Q20. What if hospital staff create fake patient records using real Ayushman Card numbers to file fraudulent treatment claims?

➤ NHA's AI claims analysis detects anomalies — like claims for patients who were never physically admitted, duplicate procedures, or treatments inconsistent with the patient's age/gender. Whistleblowers from within the hospital can also report to 14555. Hospitals found guilty face immediate delisting, recovery of fraudulent payments, and criminal prosecution.

Q21. What if misinformation spreads on WhatsApp that PM-JAY cards are being cancelled and people must re-register by paying ₹500?

➤ This is a known type of social media health scheme scam. Outreach workers should monitor community WhatsApp groups and counter false messages immediately with official clarifications. Direct people to verify at 14555 or pmjay.gov.in. Government never cancels cards en masse or charges re-registration fees.

Why Does This Exist... (Reasoning Based)

Q22. Why does PM-JAY need dedicated Ayushman Mitras instead of relying on existing hospital staff for outreach?

➤ Existing hospital staff are busy with medical duties and have no specific training or incentive for PM-JAY navigation. Ayushman Mitras are specifically trained for this role, have time dedicated to beneficiary

assistance, and act as a bridge between the community and the hospital system — a function regular staff can't reliably perform.

Q23. Why is IEC (Information, Education, Communication) material produced in regional languages instead of only Hindi or English?

➤ India's beneficiaries are predominantly rural and speak diverse regional languages. Hindi and English reach only a fraction of the target population. Regional language materials — and even pictorial content for low-literacy groups — dramatically improve comprehension, trust, and action rates among the people who most need the scheme.

Q24. Why does outreach target women specifically even though PM-JAY covers the whole family?

➤ Women are the primary healthcare decision-makers for families in India. If a woman knows about and trusts PM-JAY, she is more likely to ensure the family uses it when needed. Women's SHG networks also provide a trusted, community-embedded channel for spreading accurate information rapidly.

Q25. Why does PM-JAY outreach partner with ASHA workers instead of only using dedicated PM-JAY staff?

➤ ASHA workers already have deep trust and relationships in their villages, regular household contact, and knowledge of local health needs. Using them for PM-JAY outreach leverages an existing, trusted, community-embedded network far more effectively than deploying new external workers who communities don't know.

Q26. Why is it important to conduct outreach at construction sites, markets, and transport hubs in cities?

➤ Urban poor — migrant workers, street vendors, domestic workers — don't have fixed community structures like village panchayats. They are scattered and mobile. Outreach at their daily gathering points (worksites, markets) is the only practical way to reach them where they are, rather than expecting them to come to government offices.

Compare This vs That... (Analysis Based)

Q27. Compare PM-JAY outreach effectiveness in rural areas vs urban slums.

➤ Rural: more structured (panchayat, ASHA network), geographically concentrated, SECC data reliable. Challenges: physical distance, connectivity. Urban slums: higher density but more mobile population, higher distrust of government, language diversity (migrants), informal settlements not always in SECC. Urban outreach needs more repeated contact and community-level trust-building than rural.

Q28. Compare outreach through mass media (TV, radio) vs door-to-door outreach for PM-JAY.

➤ Mass media: reaches millions at once, cost-effective per person, builds general awareness but cannot personalise or verify eligibility. Door-to-door: expensive, slower, but creates two-way interaction, resolves individual doubts, can complete registration on-spot. Best results come from combining both — mass media creates awareness, door-to-door converts awareness into registration.

Q29. Compare outreach performance between states with high PM-JAY utilisation vs low utilisation — what are the key differences?

➤ High utilisation states: strong SHA, well-trained Mitras, active monitoring dashboards, hospital staff trained, regular camp drives, integration with ASHA incentives. Low utilisation states: passive SHA, Mitra vacancies unfilled, hospital-side resistance, no feedback loop, outreach limited to registration drives without utilisation follow-up. The difference is not scheme design but implementation quality.

Q30. Compare the effectiveness of SMS-based outreach vs in-person camp-based outreach for PM-JAY.

➤ SMS: zero marginal cost, instant reach at scale, works 24/7, but one-way communication, ignored by many, requires phone literacy. In-person camps: high cost, limited reach per day, but two-way

interaction, resolves confusion on spot, achieves immediate registration. SMS is good for reminders and confirmations; camps are essential for first-time registration and trust-building.

Summary

Stage	Consequence	Decision	Process	Fraud	Reasoning	Compare
Eligibility & Identification	8 Qs	6 Qs	3 Qs	4 Qs	4 Qs	3 Qs
Registration & Card Generation	8 Qs	6 Qs	4 Qs	5 Qs	4 Qs	3 Qs
Awareness & Outreach	6 Qs	6 Qs	4 Qs	5 Qs	5 Qs	4 Qs
TOTAL: 108 Alternative Questions across 3 Stages × 6 Thinking Styles						